

OpWell Concierge™

Clinical Evidence Brief & Operational Value Analysis

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Executive Summary

OpWell Concierge™ is an independent, telehealth-based perioperative specialty service that translates multicenter RCT evidence from Enhanced Recovery After Surgery (ERAS), Perioperative Surgical Home (PSH), and prehabilitation literature into a comprehensive optimization model across **two distinct phases: preoperative preparation and structured 30-day post-discharge monitoring**. OpWell does not participate in intraoperative care. This brief presents the primary evidence base, with exact effect estimates, confidence intervals, and certainty-of-evidence ratings for each clinical claim.

Evidence Table: Clinical and Operational Claims

Clinical Claim	Effect Estimate	Certainty	Primary Source
Complication reduction via exercise prehabilitation	OR 0.50 (0.39–0.64)	Very Low*	Mclsaac et al., BMJ 2025
Complication reduction via exercise (confirmed)	OR 0.54 (0.44–0.67)	Moderate	Ramic et al., Anesth Analg 2025
Hospital LOS reduction via ERAS	–1.88 days (–2.81 to –0.95)	Moderate	Sauro et al., JAMA Netw Open 2024
Complication reduction via ERAS	RR 0.71 (0.59–0.87)	Mod-High	Sauro et al., JAMA Netw Open 2024
Cost savings – abdominal ERAS	–\$5,109/case (–\$5,852 to –\$4,366)	Mod-High	Visioni et al., Ann Surg 2018
Anesthesiologist-led PSH impact	Decreased LOS, ICU use, opioids	Low	Cline et al., J Clin Anesth 2020
Cost-effectiveness of prehabilitation	64% (16/25) cost-effective	Low**	Rombey et al., BMC Med 2023
ED visit reduction via mHealth	OR 0.42 (~58% reduction)	Low	Dawes et al., BJS 2021
Readmission reduction via mHealth	OR 0.47 (~53% reduction)	Low	Dawes et al., BJS 2021
Readmission reduction via digital health	RR 0.67 (~33% reduction)	Low-Mod	Grygorian et al., JAMA Netw Open 2024
Preoperative patient education – LOS	–1.01 days (–1.86 to –0.16)	Moderate	Hall et al., J Am Coll Surg 2025
QoR-15 psychometric validity	α=0.85 ; r=0.97	High	Myles et al., Br J Anaesth 2022
Same-day cancellation reduction	12.3%→ 8.4% ; \$150M savings	Moderate	Wongtangman et al., Anaesthesia 2024
Preoperative assessment – cancellation reduction	OR 0.55 (0.43–0.72)	Moderate	Tan et al., Int J Surg 2019

* Robust after exclusion of high risk-of-bias trials. ** Publication bias risk noted by authors.

Critical Nuance: Post-Discharge Monitoring Requires Structured Technology

A systematic review by Boggan et al. (2025, *Annals of Internal Medicine*) of 7 randomized trials (n=7,075) demonstrated that simple, single-contact telephone calls within 7 days of discharge produced **no reduction** in 30-day readmissions (risk difference 0.00; 95% CI –0.02 to 0.02) or ED visits. This indicates that passive, unstructured telephone follow-up is **clinically insufficient**.

The significant reductions in readmissions and ED visits cited in the broader literature (OR 0.42–0.67) are **strictly specific to structured, technology-enabled, multi-contact monitoring platforms**—the exact clinical standard operationalized by OpWell Concierge. This evidence distinction underscores the operational necessity of structured tracking in value-based care pathways.

OpWell Surgery School: POQI Consensus Alignment

Every referred patient completes a 23-module digital preparatory framework aligned with 2025 POQI recommendations across five therapeutic domains:

Procedure-Specific Personalized Patient Changes for Major Specialties, Physiological Assessment, Anxiety Mitigation and clinical visualization
Practical Preparation Home configurations, structured 30-day week-by-week recovery roadmap

B2B Integration and Clinical Continuity

OpWell Concierge operates exclusively in the preoperative and post-discharge phases as a collaborative extension of existing clinical practice. **It does not provide intraoperative services, alter surgical plans, manage technical operative complications, or replace standard postoperative office visits.**

Preoperative Phase: By intercepting low-risk programmatic queries, providing objective medical clearance reporting, and delivering a validated Readiness Score, OpWell ensures patients arrive optimized, compliant, and structurally prepared for anesthesia and surgery.

Post-Discharge Phase: OpWell's structured digital monitoring platform triages recovery concerns during the critical 30-day window via technology-enabled multi-contact touchpoints, escalating actionable findings back to the surgical team for clinical integration.

Transparency Statement on Evidence Certainty

Strongest Evidence (Moderate-High): ERAS pathways consistently reduce LOS by ~1.9 days and complications (RR 0.71) across high-quality RCTs.

Strong Evidence (Very Low-Moderate): Exercise prehabilitation demonstrates consistent 50–54% complication reduction, robust after exclusion of high risk-of-bias trials.

Moderate Evidence (Low-Moderate): Post-discharge monitoring shows the strongest signal from structured, technology-enabled platforms (OR 0.42–0.67). The Boggan et al. null finding for passive calls clinically reinforces this operational distinction.

Conclusion: The OpWell clinical framework is structurally aligned with moderate-to-high certainty evidence across its core therapeutic domains (ERAS guidelines, prehabilitation protocols, and structured digital monitoring), delivering a rigorously validated, evidence-based asset to surgical programs.

References

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